

Altered Mental Status

History

- How is mental status altered? (Unconscious? Confused? Stroke symptoms? Sleepy? Agitated?)
- When did symptoms start? What was patient doing when mental status became altered?
- What is patient's normal mental status?
- Has patient had fever? Vomiting? Respiratory difficulty? Complaint of headache or chest pain?
- What is medical history – diabetic, hypertensive, stroke, substance abuse?
- Recent trauma?

Physical Exam

- Vital signs – Fever? Hypotension? Hypertension? Tachycardia? Bradycardia?
- Blood glucose – Hypoglycemic? Hyperglycemic (DKA)?
- Level of consciousness?
- Controlling airway and secretions?
- Pupils constricted or dilated?
- Signs of stroke?
- Signs of trauma?
- Signs of dehydration?

Consider Causes of Altered Mental Status:

- Infection / Sepsis / High fever
- Substance / Alcohol / Drug overdose
- Stroke / Intracranial hemorrhage
- Hypoglycemia / Hyperglycemia (DKA)
- Traumatic brain injury
- Anemia / Blood loss
- Seizures / Post-ictal
- Hypoxia
- Hypertensive Emergency
- Cardiac cause of hypotension / bradycardia

EMT/AEMT

- Follow General Medical Care Guideline
- Follow Adult Resuscitation Guideline, as indicated
- Treat Hypoglycemia if indicated - see Diabetic Emergency Guideline
- Consider altered level of consciousness as manifestation of Stroke or Head Trauma and initiate transport to appropriate facility

Paramedic

- Advanced airway management if needed, with capnography
- 12 – Lead EKG